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Perspective

Measuring quality of care for universal health coverage in the Western Pacific

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Abstract

To deliver on the promise of universal health coverage, regions and countries need to invest in high-quality health systems and put quality of care at the forefront of priority setting and agenda. The Western Pacific Region and its member countries should establish measurements and standards to ensure delivery of high-quality services. In order to effectively do so, individual and system capacities to measure and use quality-of-care data should be strengthened, localized initiatives to develop quality-of-care measures of practice and policy relevance need to be carried out, accountability mechanisms should be in place, and the role of quality as a driver of better health outcomes and economic benefits should be emphasized among all stakeholders. Such initiatives will ensure that quality-of-care measures are "measurements for action" toward quality improvement and reform.

Keywords: quality of care; universal health coverage; Philippines; Western Pacific

The importance of quality of care and how to measure it

Quality of care is fundamental in the pursuit of universal health coverage (UHC) and the Sustainable Development Goals [1,2]. Its importance has been reaffirmed through the United Nations (UN) Political Declaration on UHC and other global development priorities and agenda. Despite being a key focus area, however, around 8.4 million people in low- and middle-income countries (LMICs) continue to die every year because of poor quality of care resulting in US$ 6 trillion in economic losses [1,2]. This highlights the need to invest in patient-centered health systems that deliver high-quality services to ensure the efficient use of limited health resources and save millions of lives from preventable diseases and mortality [1,2].

Recognizing that meaningful and accurate measurements are central to high-quality health systems, global and international efforts have been made to develop measurements of quality. These include the Healthcare Access and Quality Index based on amenable mortality [3] and a set of indicators proposed in the World Health Organization (WHO) monitoring and evaluation framework for primary health care [4]. At the regional level, the WHO Africa Region has constructed a multidimensional index for their setting, while the Eastern Mediterranean Region has identified their priority indicators [5,6]. Many others have been developed, but regions and countries need to lead localized initiatives to ensure appropriateness to the setting, responsiveness to needs, acceptability to users, and usability for quality improvement. Extensive collection and reporting of indicators, as in the experience of Mexico with more than 1000 collected [2], prove to be inefficient and ineffective in providing and acting on evidence-informed recommendations.

Progress and challenges in the Western Pacific: illustrative example from the Philippines

In the region, quality healthcare has been articulated as an aspiration in national health policies for UHC, acknowledging that poor-quality health services contribute to health system inefficiencies and poor health outcomes [7]. For 2020–2025, the region's vision "For the Future" is explicit on its commitment to quality healthcare as it relates to patient safety and combating antimicrobial resistance. However, it is viewed as a secondary priority to more primordial issues such as availability, service delivery, and financing [7,8]. This is reflected in the absence of a regional quality-of-care index, with various indicators and methodologies applied depending on country priorities, capacities, and resources.

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The Philippines, an archipelagic LMIC, continues to address issues on service delivery and financing, but now needs to put quality of care at the forefront with its transition toward UHC and primary health care. Numerous data sources report on quality at the national, subnational, community, facility, or individual level. However, quality measurements are fragmented by disease and not standardized, owing to (i) gaps in capacities to develop indicators that are of high impact and value and (ii) the lack of a common understanding of quality among service providers, patients, and political actors. There are also limited data on experiential quality or patient experience, as current systems focus on service availability, selected processes, and health outcomes. More importantly, findings are not translated into strategies and policies for improved quality healthcare, even with established accreditation and regulatory standards. This is partly due to the absence of a uniform set of measures in both public and private facilities [9], embedded within accountability mechanisms and legal frameworks. Additionally, health information systems are siloed—many of which in public primary care facilities in remote settings remain paper-based and error-prone—and therefore lack transparency, interoperability, usability, and scalability. This is further aggravated by the decentralized Philippine health system, which makes routine collection of indicators and storage of large volumes of data difficult especially in resource-constrained settings. Collectively, these prevent comprehensive, comparable, and regular measurements for UHC monitoring and healthcare reform across geographical settings and population groups.

Recommendations

For quality-of-care measures to be "measurements for action" [2], the region and the countries themselves will need to determine their highest priorities and make the necessary investments. The development of metrics may be done through localized initiatives to establish measures that allow for more granular and context-specific estimates, and to identify appropriate equity stratifiers (i.e., demographic, social, economic, racial, or geographic characteristics that identify population subgroups, and detect disparities within and across countries). These approaches can lead to a better understanding of sources of variations in quality, while ensuring that their routine collection does not place undue burden on information systems and stakeholders [5]. Furthermore, quality-of-care data should be centered on people and their experiences and perceptions of the health system and service delivery, ideally collected throughout the patient pathway [10]. Health information systems especially in low-resource settings should be strengthened toward the following key features for timely and cost-efficient assessments: (i) comprehensive to capture inputs, processes, and outcomes including experiential quality and patient-care-sensitive measures; (ii) interoperable to integrate data across different systems and platforms; and (iii) responsive to account for evolving priorities and needs over time. To effectively drive action and reform, measurements and performance accountability mechanisms should be aligned and institutionalized through a national policy on health system quality [2]. All key stakeholders including patients and the public, community health volunteers, health professionals and managers, researchers, and those from the civil society, the private sector, and the government should be informed of the importance of measuring quality of care as a necessary input to quality health systems. More importantly, they should be involved in creating a supportive environment for quality healthcare using a multisectoral approach with strong governance structures. In doing so, we develop high-quality health systems that are patient-centered, responsive to a broad array of health conditions, cost-efficient, sustainable, and socially inclusive and accountable.

Author Contributions

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Data availability

All data cited in this article are publicly available.

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